

Immunotherapy for bladder cancer

Immunotherapy uses the immune system to find and attack cancer cells. A type of immunotherapy called a checkpoint inhibitor may be used to treat bladder cancer that has spread.

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About immunotherapy for bladder cancer

The immune system protects the body against illness and infection. Immunotherapies are treatments that use the immune system to find and attack cancer cells.

A type of immunotherapy called a checkpoint inhibitor may be used to treat [bladder cancer](#) that has spread.

Checkpoint inhibitors are usually only used if:

- cisplatin [chemotherapy](#) is not suitable
- cisplatin chemotherapy is not working well
- the cancer comes back after chemotherapy.

Sometimes, they are given after chemotherapy to keep the cancer under control. This is called maintenance therapy.

Your cancer doctor will explain if a checkpoint inhibitor is a suitable treatment for you. Some drugs may only be available as part of a clinical trials.

Learn more about taking part in a [cancer clinical trial](#).

Checkpoint inhibitors that are used to treat bladder cancer include:

- [atezolizumab](#)
- [pembrolizumab](#)
- [avelumab](#)
- [nivolumab](#)
- [ipilimumab](#)
- [durvalumab](#)
-

tremelimumab.

Your cancer doctor will tell you if they think any of these drugs, or a drug not listed, is suitable for you.

Immunotherapies for non-muscle-invasive bladder cancer

A different type of immunotherapy called BCG treatment is used to treat non-muscle-invasive bladder cancer. This is cancer that is in the inner lining or the connective tissue that surrounds the inner lining of the bladder.

BCG is short for Bacillus Calmette Guérin. BCG treatment is given directly into the bladder (intravesical). It is used to treat high-risk non-muscle-invasive bladder cancer and sometimes intermediate-risk bladder cancer.

We have separate information about [BCG treatment for bladder cancer](#).

New treatments are being developed for non-muscle-invasive bladder cancer. This includes immunotherapy into a vein (IV). This is only available as part of a clinical trial. Your doctor will give you more information about this if they think it may be useful for you.

How checkpoint inhibitors are given

You have the immunotherapy drug as a drip into a vein (intravenously). You usually have treatment in the chemotherapy day unit and go home afterwards.

Your cancer doctor or nurse will explain your treatment plan and what to expect. After the treatment, you have a rest period of a few weeks. Together, the immunotherapy and the rest period are called a cycle of treatment. The length of a cycle depends on the treatment you are having.

Possible side effects

Common side effects of checkpoint inhibitor drugs include:

- feeling tired
- skin changes
- diarrhoea
- feeling short of breath
- hormone problems.

Side effects may be mild, but sometimes they can become serious very quickly if they are not treated. Side effects can start weeks, months, or sometimes more than a year, after you finish treatment.

Your cancer doctor or nurse will give you information about managing side effects. It is important to tell them about any side effects you have, even after you have stopped having treatment.

Your doctor, nurse or pharmacist will also give you a card with information about your treatment on it. It is important to always have the card with you during treatment and after treatment finishes. You should show it to any doctor or healthcare professional you need to see.

About our information

References



Below is a sample of the sources used in our bladder cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Mottet N, Bellmunt J, Briers E, et al. Non-muscle-invasive bladder cancer (TaT1 and CIS). European Association of Urology (Internet), 2021. Available from uroweb.org/guideline/non-muscle-invasive-bladder-cancer (accessed September 2021).

Witjes JA, Bruins HM, Cathomas R, et al. Muscle-invasive and metastatic bladder cancer. European Association of Urology (Internet), 2021, Available from uroweb.org/guideline/bladder-cancer-muscle-invasive-and-metastatic (accessed September 2021).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

Reviewed: 01 November 2022 | Next review: 01 November 2025

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information.](#)

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help